

Tinea Capitis

Like fluconazole, hepatotoxicity occurs at much lower rates with itraconazole than with ketoconazole. However, there are reports of hepatic failure in patients treated with itraconazole. Itraconazole has also rarely been linked to congestive heart failure.

Terbinafine (See Chap. 233)

Doses of 3 to 6 mg/kg/day of terbinafine can cure *Trichophyton* tinea capitis in 2 to 4 weeks, but 4 to 8 weeks of treatment are required for eradication of *Microsporum*. A randomized comparison of 4 weeks of terbinafine to 8 weeks of griseofulvin showed no statistically significant difference overall. However, a subgroup with *Trichophyton* infection responded better to terbinafine, while *M. audouinii* infections responded better to griseofulvin.

Terbinafine also produces gastrointestinal side effects and, rarely, hepatitis. Although it has fewer cytochrome P_{450} effects than other oral antifungals, tricyclic antidepressant toxicity has occurred with concomitant terbinafine use due to CYP2D6 inhibition. Like itraconazole, there are reports of liver failure in patients using this drug.

Adjuvant Therapy (See Chap. 219)

A number of shampoos have been shown to help eradicate dermatophytes from the scalp of children, and their use is recommended by most experts. Selenium sulfide (1% and 2.5%), zinc pyrithione (1% and 2%), povidone iodine (2.5%), and ketoconazole (2%) are all commonly used. Common recommendations are to use these shampoos 2 to 4 times weekly for 2 to 4 weeks.

With markedly inflammatory tinea capitis, oral glucocorticoids may reduce the incidence of scarring. Although there is no consistent evidence for differences in cure rates, oral glucocorticoids are beneficial in relieving pain and swelling. The usual dose of prednisone is 1 to 2 mg/kg each morning for the first week of therapy.

Household transmission may be prevented through treatment of infected family members or animals and disinfection of the environment. The use of ketoconazole 2% shampoo or selenium sulfide 2.5% by all household members three times weekly also reduces transmission by decreasing spore shedding.

Tinea Barbae

Like tinea capitis, oral antifungals are essential in the treatment of tinea barbae. Griseofulvin at 1 g daily, itraconazole 200 mg daily for 2 to 4 weeks, terbinafine 250 mg daily for 2 to 4 weeks, or fluconazole 200 mg daily for 4 to 6 weeks are all effective treatment plans. Inflammatory infections caused by *T. verrucosum*, *T. mentagrophytes*, and *M. canis* usually resolve without treatment in 2 to 4 weeks. Systemic glucocorticoids are helpful for severe inflammatory lesions.

Tinea Corporis / Tinea Cruris

For isolated lesions on glabrous skin, topical agents such as allylamines, imidazoles, tolnaftate, butenafine, or ciclopirox are effective. Most are applied twice daily for 2 to 4 weeks. Oral antifungals are reserved for widespread or more inflammatory lesions.

Comparative studies in adults show that fluconazole (150 mg weekly for 4 to 6 weeks), itraconazole (100 mg daily for 15 days), and terbinafine (250 mg daily for 2 weeks) are as effective as griseofulvin (500 mg daily for 2 to 6 weeks), with no significant differences in adverse events.

Safe and effective regimens for children include ultramicrosize griseofulvin (10 to 20 mg/kg/day for up to 6 weeks), itraconazole (5 mg/kg/day for 1 week), and terbinafine (3 to 6 mg/kg/day for 2 weeks).